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SYMPTOMS AND TREATMENT FOR OVERDOSAGE AND ANTIDOTE(S):

Symptoms:

Clumsiness or unsteadiness or dryness of mouth, nose or throat, flushing or redness of face, shortness of breathing (due to antimuscarinic effect).
Hallucination or trouble in sleeping, ataxia, incoordination athetosis, convulsions leading to coma and death (due to CNS stimulation effect)
CNS depression (severe drowsiness), hypotension (feeling faint) and cardiac arrhythmias (fast or irregular heartbeat).

Treatment:

No specific therapy or antidote, treatment is symptomatic and supportive with possible utilization of the following:
Induction of emesis (syrup of ipecac recommended); Gastric lavage if patients is unable to vomit within 3 hours of ingestion; Saline cathartics and activated charcoal, if appropriate; Vasopressors (except epinephrine) to treat hypotension;

PACKING / PACK SIZES:

Blister pack of 10 tablets, 10 x 10's / box and 100 x 10's / box;

STORAGE CONDITIONS, USER INSTRUCTIONS AND PHARMACEUTICAL PRECAUTIONS:

Store below 30°C in a well-closed container.

User instructions.

This drug may cause drowsiness and dulling of mental alertness. Until the extent of this effect is known, caution should be taken when driving, operating machinery, or performing other tasks requiring mental alertness.

Patients should be abstained from alcohol and other drugs which cause drowsiness.

SHELF LIFE:

3 Years

MANUFACTURER AND PRODUCT REGISTRATION HOLDER

SM PHARMACEUTICALS SDN BHD (218620-M)

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SM PHARMACEUTICALS SDN. BHD.

SENSITAMINE TABLET

(CHLORPHENIRAMINE MALEATE TABLET 4 MG)

DESCRIPTION:

Pink, round and uncoated tablets with break line on one side.

COMPOSITION:

Each tablet contains: Chlorpheniramine maleate 4 mg.

ACTIONS AND MODE OR MECHANISMS OF ACTION:

Antihistamine used in the treatment of allergy acts by competing with histamine for H₁-receptor sites of effector cells. It thereby prevents but does not reverse, responses mediated by histamine alone. The relationship between chemical structure and antihistamine activity is highly complex, but most of the antihistamines have the core structure of ethylamine - CH₂-CH₂N = which is also present in histamine. The antimuscarinic actions of most antihistamines provide a drying effect on the nasal mucosa.

PHARMACOLOGY:

Chlorpheniramine maleate is an alkylamine derivative and one of the most potent antihistamines and generally causes less sedation than promethazine. Besides blocking the effect of histamine on H₁ receptor, chlorpheniramine also has other effects which include anticholinergic effects, adrenaline enhancing and / or adrenaline antagonising, and serotonin antagonising effects. Chlorpheniramine may also have a slight local anaesthetic effect. Chlorpheniramine does not abolish the effect of histamine on H₂ receptor.

In general, Chlorpheniramine is readily absorbed from the gastrointestinal tract. It is widely distributed throughout the body tissue and appears to be subjected to enterohepatic circulation. About 70% is bound to plasma protein. The plasma half-life after oral administration is reported to be 12 to 15 hours. Chlorpheniramine maleate is extensively metabolised to form polar and non-polar metabolites; 2 metabolites have been identified as a result of N-demethylation and these are monodesmethyl chlorpheniramine and didesmethyl chlorpheniramine. About 35% of a dose is excreted in the urine in 48 hours. The excretion of unchanged drug accounts for about 3 - 10 % of the doses but it is increased by an increased urinary flow. Both demethylated metabolites are excreted in the urine. About 10% of a dose is excreted in the faeces.

TOXICOLOGY:

Cross sensitivity

Patients intolerant of one antihistamine may be intolerant of other antihistamines.

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Pregnancy / reproduction

Safety for the use of Chlorpheniramine Maleate during pregnancy has not been established, it should only be used during pregnancy when clearly needed and when potential benefits outweigh the potential unknown risks of the foetus. Use during third trimester may result in reactions in the newborn or premature neonates.

Breast-feeding

Small amounts of antihistamines are excreted in breast milk, use is not recommended in nursing mothers because of the risk of adverse effects, such as usual excitement or irritability of antihistamines in infants.

Antihistamines may inhibit lactation because of their antimuscarinic action.

INDICATIONS:

1. Symptomatic treatment of perennial and seasonal allergic rhinitis, vasomotor rhinitis and allergic conjunctivitis due to inhalant allergens and foods.
2. Symptomatic treatment of pruritus associated with allergic reactions and of mild, uncomplicated allergic skin manifestation of urticaria and angioedema and in dermatographism.
3. May be used in combination with a decongestant (Pseudoephedrine) for the temporary relief of nasal and sinus congestion associated with the common cold and allergic rhinitis.

CONTRAINDICATIONS:

In newborn and premature infants, during lactation, as treatment for asthma, in patients on monoamine oxidase (MAO) inhibitors. Also see 'TOXICOLOGY' and 'PRECAUTIONS / WARNINGS'.

SIDE EFFECTS / ADVERSE REACTIONS:

Sedation, mild as compared to promethazine; Gastrointestinal disturbances - nausea, epigastric pain, and rarely vomiting, diarrhoea or constipation; Blurred vision or changes in vision, confusion and dizziness are less common; Dryness of mouth, nose or throat and difficulty in micturition due to anticholinergic effect may also occur; Loss of appetite; Increased skin sensitivity to sunlight; Blood dyscrasias (rare); Thickening of mucous, fast heartbeat, paradoxical reactions (nightmares, unusual excitement, nervousness, restlessness or irritability) ringing or buzzing in the ears and skin rash.

PRECAUTIONS / WARNINGS:

All antihistamines given in effective dosage, may cause dizziness or drowsiness. In consequence, until the effect of treatment is known, patients treated with chlorpheniramine should be warned not to take charge of vehicles or machinery. The sedative action of alcohol may be potentiated by antihistamines so that drowsiness can occur in patients not otherwise subjected to it if alcohol is taken.

Risk-benefit must be considered when the following medical problems exist: bladder neck obstruction, prostatic hypertrophy (symptomatic), urinary retention, angle closure glaucoma (or predisposition to), open angle glaucoma or sensitivity to chlorpheniramine maleate.

Paediatrics: Use is not recommended in newborn and premature infants because this age group has an increased susceptibility to anticholinergic side effects, such as central nervous system (CNS) excitation, and an increased tendency towards convulsion. A paradoxical reaction characterized by hyperexcitability may occur in older children taking antihistamines.

Geriatric: Dizziness, sedation, confusion and hypotension may be more likely to occur in geriatric patients taking antihistamines. A paradoxical reaction characterized by hyperexcitability may occur in geriatric patients taking antihistamines. Geriatric patients are especially susceptible to the anticholinergic side effects, such as dryness of mouth and urinary retention (especially in males), of the antihistamines. If these side effects occur and continue or severe, medication should probably be discontinued.

Dental: Prolonged use of antihistamines may decrease or inhibit salivary flow, thus contributing to the development of caries, periodontal disease, oral candidiasis and discomfort.

DRUG INTERACTIONS:

1. Alcohol and CNS depression - producing medications - may potentiate the CNS depressant effects.
2. Anticholinergics or other medication with anticholinergic activity (including maprotiline and tricyclic antidepressant) - anticholinergic effects may be potentiated.
3. Ototoxic medications - symptoms of ototoxicity such as tinnitus, dizziness or vertigo may be masked.
4. Photosensitizing medications - concurrent use with antihistamine may cause additive photosensitising effects.
5. Phenytoin - chlorpheniramine inhibits phenytoin metabolism and can lead to phenytoin toxicity.

LAB INTERACTION:

Chlorpheniramine may interfere with skin tests using allergen extracts.

RECOMMENDED DOSAGE, DOSAGE SCHEDULE AND ROUTE OF ADMINISTRATION:

Usual adult dose - Oral, 4 mg every four to six hours as needed.

Usual adult prescribing limit - Up to 24 mg daily.

Usual paediatric dose - Children up to 6 years of age: use is not recommended.

Children 6 years to 12 years: Oral, 2 mg, three or four times a day as needed; not to exceed 12 mg per day.

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